SUPPLEMENTARY DIGITAL MATERIAL 3

Supplementary Table III.—Characteristics of aim, treatment, type of rehabilitation, assessment tools and result of the included studies.[34-54]

Author and year	**Aim of the study**	**Treatment**	**Type of rehabilitation**	**The frequency of rehabilitation**	**Duration of therapy**	**Assessment**	**Result**
Yu J *et al.* 2019 [34]	Assessment of the effectiveness of intensive rehabilitation in improving the neurological deficit after BT surgery in relation to the group of patients after stroke	Surgery resection- 13 Surgery resection+chemotherapy- 3 Surgery resection+radiotherapy- 11 Surgery resection+chemotherapy + Radiotherapy- 8	PT: range of motion exercises, resistance exercises, balance training, endurance training, gait training. OT: cognitive improvement, proprioceptive neuromuscular stimulation of the upper limbs and ADL exercises	5 times a week for 1 hour of PT and 1 hour of OT	mean 37.2±11.9 days	FMA, BBS, Korean version of the Modified BI Among patients with BT of the ECOG to confirm the functional level.	Significant improvement in FMA, BBS, Korean version of the Modified BI, BT and stroke groups
Hojan K *et al.* 2020 [35]	Clinical observation concerning assessment of the effectiveness of the 12-week comprehensive inpatient or outpatient	Radiotherapy- 25 Chemotherapy - 13	Inpatient group: PNF Bobath therapy, coordination, balance and gait treatment, as well as endurance training. In addition,	Inpatient group: 6 days/week for 150 min/day Outpatient group: 6	12 weeks	The Functional Independence Measure (FIM), The Barthel Index (BI), The	Patients in both programs significantly improved their physical functioning scores

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
	rehabilitation treatment carried out as prevention against functional disability in BT patients with various degrees of disability after the end of oncological treatment		those patients had 1 h/day of individual OT, neuropsychological therapy, and speech therapy 5 days/week. Outpatient group: one hour of individual therapy with neuro-muscular reeducation exercises and one hour of group exercises using different devices such as cycling, treadmill, Pilates, yoga and general fitness exercises. In addition, patients had the opportunity to consult a psychologis	days/week for 150 min/day		Berg Balance Scale (BBS), FACT	in daily activities

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			t, a social worker, and an OT (5 days/per week)				
Han E Y *et al.* 2015 [36]	To identify the functional improvement in BT patients after 4-week conventional rehabilitation therapy and to compare the cognitive impairment of BT patients with that of subacute stroke patients using computerized neuropsychological testing and determine the effects on functional outcomes of daily activity	-	PT by NDT therapists, and neuromuscular electrical stimulation therapy and aerobic exercise were also applied. In addition, patients also received OT for stretching and strengthening exercises of the upper extremity and task-oriented therapy for ADL, fine motor training, and sensory motor recovery.	1 hour per a day	4 weeks	The Motricity Index was used to assess motor impairment, The Korean version of the Modified BI was used to measure the functional status of patients in activities of daily living	All BT and stroke patients significantly improved after 4 weeks of rehabilitation treatment.

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
Geler-Kulcu D *et al.* 2009 [37]	To examine the functional outcomes of patients with BT compared to patients with stroke by using instruments that include specific tasks related to locomotor function.	Surgery- 21	PT focused on positioning, postural control, range of motion and progressive resistive exercises together with endurance and gait. Elements of Brunnstrom's movement therapy, Bobath neurodevelopmental treatment and PNF. OT if needed.	5 time/week for 1 hour	mean number of sessions 11.5±5.8	Motor Assessment Scale, Postural Assessment Scale for Stroke, BBS, FIM	Both groups improved significantly their mobility scores Motor Assessment Scale, Postural Assessment Scale for Stroke, BBS, FIM
Greenberg E *et al.* 2006 [38]	Examination of functional improvement and outcome in patients with primary BT and to compare them with those of	None of the patients received adjuvant therapy for their tumor during inpatient rehabilitation.	Inpatient rehabilitation	-	mean length of stay in inpatient rehabilitation (days): meningiomas-24	FIM instrument	BT patients with both gliomas and meningiomas hospitalized for inpatient rehabilitation improved their FIM ratings

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
	stroke patients				gliomas - 23 stroke- 75		after a short inpatient rehabilitation program.
Bilgin S *et al.* 2014 [39]	Description and comparison of the functional outcomes following the acute rehabilitation in patients with BT or brain injury	None of the patients with BT received radiation therapy or chemotherapy through rehabilitation.	NDT	30 minutes in a day and 5 days in a week.	median for BT patients- 6 (1-52) days and for brain injury 20.5 (2-123) days	The BI was used to determine the functional status at the pre- and post-rehabilitation.	Functional levels of patients with BT and brain injury increased following the early rehabilitation.
Yoon J *et al.* 2015 [40]	Evaluation of the ability of VR-based rehabilitation to improve hand, arm, and shoulder function in patients with BT.	Resection- 17 Resection+ chemotherapy- 10 Resection+radiation- 9 Gamma Knife Radiation Surgery- 3 Radiation only- 1	Conventional OT consisted of range of motion exercises, fine motor training, and strengthening upper extremities exercises. Each exercise took approximat	IG: VR program 30 mins per session for 9 sessions and conventional OT 30 mins per session for 6 sessions	3 weeks	The Box and Block test, the Manual Function test, FMA scale The Korean version of the Modified BI	Each group exhibited significant posttreatment improvements in the Box and Block test,Manual Function test, FMA, and Korean

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			ely 10 mins. VR-based rehabilitation was performed by an OT proficient in the use of the Interactive Rehabilitation and Exercise System. Six VR programs (Birds and Balls, Conveyor, Drums, Juggler, Coconuts, and Soccer) were selected.	CG received conventional OT alone 30 mins per session for 15 sessions			version of the Modified BI
Huang M E *et al.* 2000 [41]	Demonstration that people with BT can achieve comparable functional benefits compared to people with traumatic brain injury, justifying the need	-	Inpatient rehabilitation	-	22 weeks	FIM	Significant gains in FIM change were seen when comparing admission to discharge FIMs within each group

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
	for inpatient rehabilitation						
Mukand J A *et al.* 2001 [42]	Discuss the frequency of neurologic deficits and functional outcomes in a random population of patients with BT.	Surgery 28 Surgery and XRT 7 Surgery and chemotherapy and XRT 6 XRT and chemotherapy 4 XRT 4 Chemotherapy 4 No treatment 1 chemotherapy 0	Inpatient rehabilitation	-	mean 19.7 days	FIM	Patients with BTs exhibited significant functional gains after inpatient rehabilitation, as shown by the increase in their FIM scores
Huang M E *et al.* 1998 [43]	Examination of functional improvement and outcomes in patients with BT.	-	-	-	mean 33.7 days	FIM	BT patients have functional gains comparable to those of stroke patients with similar

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
							functional status on rehabilitation admission, often in a shorter period
Hansen A *et al.* 2018 [44]	Assessment of the safety and feasibility of using an interdisciplinary rehabilitation intervention for a future randomized controlled trial in patients with gliomas in the initial treatment phase.	Gross resection-23 Chemoradiation- 20	The exercise comprised: 5–10 minutes of initial warm-up; cardio-training of 20 minutes of cycling or treadmill with intensities ranging from 65% to 85% of the heart rate reserve monitored by pulse; strength training of main extremity muscle groups with increasing load ranging	PT 90 minutes 3 times a week OT 60 minutes 2 times a week	12 weeks (6 weeks supervised training and 6 weeks unsupervised training)	BBS; a 10-meter walking test; a Watt-Max cycle test; and a 3–8 repetition maximum test for estimating maximum muscle strength by Brzycki's equation EORTC-QLQ-C30 and BN20	The study has shown that an intensive rehabilitation intervention of PT and OT in the initial treatment phase in patients with gliomas whose KPS is ≥70 are safe and feasible, if relevant inclusion criteria and precautionary screening are made.

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			from 15 to 10 repetition maximum; and body awareness training or relaxation tailored to personal needs. The OT focused on improving patients' functioning, activity and participation levels by adapting activities, by regaining or developing abilities to perform an activity and/or by rebuilding and developing patient skills. PT interventions included a six-week membership at a local gym with an			questionnaire Activity in patients' daily living was assessed through AMPS testing	

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			instructor where they continued to follow the exercise protocol, with resistance training being increased to 8 repetitions maximum. Patients attending OT received a weekly 20-min telephone-guided intervention during which they continued working towards the initiated activities.				
Baima J *et al.* 2017 [45]	Evaluation of compliance with and safety of a novel independent home exercise program for patients	Brain surgery-9 Chemotherapy - 7 Radiotherapy-1	The home exercise program. The program included a set of resistance band exercises for	Every day	1 months	FACT-BR	Subjects with high-grade BT demonstrated compliance with and safety of a novel independe

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
	with high-grade BT		strengthening muscles used in walking (deltoids, quadriceps, and hip abductors), a balance exercise, and recommendations for walking. The balance exercise was standing on a single leg while holding onto a chair as needed for balance. The walking recommendations were to walk at one's own pace for 20 min indoors or outdoors, to "not exert yourself," and to separate walking time into two sets of 10 min if 20				nt strength and balance exercise program in the home setting. Higher frequency of exercising was associated with life quality parameters as well as marriage and income.

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			min in a row was perceived as difficult.				
Roberts P S *et al.* 2014 [46]	To examine the impact of an inpatient rehabilitation program on functional improvement and survival among patients with newly diagnosed GBM who underwent surgical resection of the BT.	Rehabilitation/ Non rehabilitation group: Resection (biopsy-29/100, near/gross total resection 51/144, partial resection 20/64) Radiation 86/251 Temozolomide 77/231 Radiation and temozolomide 74/226 Vaccine 17/34 Upfront chemotherapy 5/14 Upfront Avastin 11/31	The inpatient rehabilitation program consisted of medical and functional management from an interdisciplinary team in a hospital environment (personnel from physiatry, rehabilitation nursing, PT, OT, speech-language pathology, neuropsychology, therapeutic recreation, and social work/case management were core	Patient participated in a minimum of 3 hours of therapy 5 of 7 days per week	mean 13.2 days	FIM	Overall functional improvement and survival time for patients who participated in the inpatient rehabilitation program

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
		Upfront gamma knife 10/19	team members).				
Reilly JM *et al.* 2020 [47]	To compare functional outcomes between matched patients admitted to acute inpatient rehabilitation after initial diagnosis of GBM and after diagnosis of recurrent GBM	Initial GBM: radiation-2, chemotherapy-3, surgical- 25 Recurrent GBM: radiation-7, chemotherapy-21, surgical-15, other-3	Inpatient Rehabilitation	-	Initial GBM – 16.4 days Recurrent GBM-14 days	FIM	Functional outcomes of patients admitted for acute Rehabilitation after an initial diagnosis of GBM and after recurrence of GBM were similar
Marciniak CM *et al.* 2001 [48]	To assess the extent of functional gains measured before and after inpatient rehabilitation in patients who have primary or metastatic BT, and to identify whether the	Radiotherapy-20 Chemotherapy - 4	Comprehensive inpatient rehabilitation	-	mean 24.6 days	FIM	Metastatic or primary BT type does not affect the efficiency of functional improvement during inpatient rehabilitation. Patients receiving concurrent radiation

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
	tumor type, recurrent tumor, or ongoing radiation influences outcomes						therapy make greater functional improvement per day than those not receiving radiation. Patients with recurrent tumors make significantly smaller functional motor gains than those completing inpatient rehabilitation after the tumor's initial diagnosis.

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
O'Dell M W *et al.* 1998 [49]	To document functional outcome in persons with BT undergoing inpatient rehabilitation and to compare outcomes with a group of traumatically brain injured patients.	Radiation therapy- 17,5% Chemotherapy - 12,5%	inpatient rehabilitation	-	mean 17.8 days	FIM	Daily functional gains made by persons with BT undergoing rehabilitation were similar to those made by a group of persons with TBI matched by age, gender, and admission functional status.
Ayotte S L *et al.* 2017 [50]	To investigate the feasibility of aerobic exercise training in patients with BT and its effects on aerobic function, strength, fatigue, and functional outcomes.	Resection- 13 None of the participants received adjunctive therapies during their inpatient stay	Individualized aerobic training on a recumbent cycle ergometer. Training goal was 20 minutes of continuous cycling at moderate intensity.	Phase 1: 5 days per week Phase 2: 3 days per week.	6 weeks	Aerobic capacity (graded submaximal cycling test), 6MWT, FIM, lower-extremity composite strength, and FACT–F	Statistically significant changes in all outcome measures found during phase 1, with moderate gains in aerobic capacity and the 6MWT. All

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
							participants reached the training goal of 20 minutes of continuous cycling at moderate intensity and adherence to 5 training sessions per week in phase 1. No adverse events reported.
Shahpar S *et al.* 2018 [51]	To determine if an interdisciplinary outpatient rehabilitation program will improve functional outcomes in patients with malignant BT	Chemotherapy or radiation therapy- 40 Surgical resection of their tumors- 34	PT, OT, and speech therapy, nursing and physician care, vocational rehabilitation and psychological counseling.	2.5 to 5 times a week, 3 to 6 hours a day	76.9 days (min 38; max 227 days)	The Day Rehabilitation Outcome Scale, Disability Rating Scale	There was a significant improvement in total Day Rehabilitation Outcome Scale (P<.001), mobility (P<.001), Activities of Daily Living (P<.001),

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
							and communication (P<.001). There were no significant changes over time in the Disability Rating Scale scores.
Bartolo M *et al.* 2012 [52]	Investigation the functional outcome, after an early rehabilitation program, of neurosurgical inpatients affected by primary BT, comparing them with stroke patients.	100 % surgery	1. Passive/assisted stretching exercises for the shoulder, elbow, ankle, and knee, to increase the range of motion of the affected extremities and prevent contractures; 2. Passive/assisted strengthening exercises for trunk muscles and upper and	60 min per a day; 6 time a week	4 weeks	The Sitting Balance score; The Standing Balance score; Hauser Index; Massachusetts General Hospital Functional Ambulation Classification FIM	All the measures of outcome were indicative of substantial improvements for neuro-oncological and for stroke patients

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
			lower limbs; 3. Strengthening exercises in a functional context to promote upper limb and/or hand use; 4. Balance exercises, initially in sitting position and then in standing position (once the patient had recovered the ability to stand, even with assistance); 5. Ground-floor walking (including step control) first with assistance, then under supervision.				

Author and year	Aim of the study	Treatment	Type of rehabilitation	The frequency of rehabilitation	Duration of therapy	Assessment	Result
Fu J B *et al.* 2019 [53]	Evaluate FIM changes and incidence of serious medical complications requiring return to the primary acute care service of acute rehabilitation cancer inpatients with LMD	Intrathecal Chemotherapy 5 (16.7%) Radiation 4 (13.3%) Oral Chemotherapy 2 (6.7%)	Acute inpatient rehabilitation	-	11 days	FIM	LMD patients who completed acute inpatient rehabilitation made statistically significant improvements on the majority of FIM items.
Khan F *et al.* 2014 [54]	To evaluate effectiveness of a multidisciplinary rehabilitation program for persons following definitive primary BT treatment in a community cohort.	Surgery resection-33 Chemotherapy - 51 Radiotherapy-68	intensive ambulatory multidisciplinary rehabilitation (Social, Psychology, OT and PT)	2-3 times a week	6- 8 weeks	FIM	BT survivors can improve function with multidisciplinary rehabilitation, with some gains maintained up to 6 months.

BT: brain tumor; PT: physical therapy; OT: occupational therapy; FMA: Fugl- Meyer Assessment; BBS: Berg Balance Scale; BI: Barthel Index; FIM: Functional Independence Measure; ECOG: Eastern Cooperative Oncology Group; PNF: proprioceptive neuromuscular facilitation; NDT: neurodevelopmental treatment; VR: virtual reality; EORTC-QLQ: European Organisation for Research and Treatment of Cancer Quality of Life Questionnaire; EORTC-BN20: European Organisation for Research and Treatment of Cancer Brain Neoplasm Module; AMPS: The

Assessment of Motor and Process Skills; KPS: Karnofsky Performance Status Scale; FACT: The Functional Assessment of Cancer Therapy; 6MWT: 6 Minute Walk Test.

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